

August 12, 2026

Absolute Total Care (Centene)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Laticia Dibbert (MRN MUSC9662433), Claim MUSC-20260413-E65A-1

Patient	Laticia Dibbert (DOB 1948-11-08)
MRN	MUSC9662433
Member ID / Group	ABS643321435 / GRP-92000
Claim number	MUSC-20260413-E65A-1
Date of service	2026-04-13
Procedure billed	CPT 99223 — Initial hospital care, high complexity
Primary diagnosis	E66.9 — Obesity, unspecified
Denied amount	\$2,212.20
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-08-12

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this first-level appeal on behalf of patient Laticia Dibbert (MRN: MUSC9662433, Member ID: ABS643321435) regarding the denial of Claim MUSC-20260413-E65A-1 for services rendered on April 13, 2026. The claim was submitted on April 28, 2026, and denied on May 14, 2026, under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the payer remark stating that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." The denied amount is \$2,212.20 for CPT 99223, Initial Hospital Care, High Complexity. We respectfully contend that this denial is not supported by the clinical facts of record and request full reversal and payment.

CLINICAL BACKGROUND

Ms. Dibbert is a 77-year-old female with a complex, longstanding medical history that includes chronic congestive heart failure with a documented left ventricular ejection fraction of approximately 35%, obesity, osteoporosis, anemia, prediabetes, and rheumatoid arthritis. Her cardiac condition has been actively managed with furosemide and extended-release metoprolol succinate. Her osteoporosis, confirmed by a DXA T-score of approximately -3.0, is treated with alendronic acid. She carries multiple concurrent chronic conditions, each independently requiring ongoing medical management. The April 13, 2026 inpatient admission was billed at Place of Service 21 under the care of rendering physician Priya N. Raghavan, MD (NPI 1669504132), with a primary diagnosis of Obesity, unspecified (E66.9) and secondary diagnosis R69.

BASIS FOR APPEAL

The denial rests on the assertion that the documentation does not establish medical necessity for the service as billed. We respectfully disagree. CPT 99223 describes initial hospital care requiring a comprehensive history, comprehensive examination, and medical decision-making of high complexity. Given Ms. Dibbert's active comorbidity burden — most critically, chronic congestive heart failure with significantly reduced ejection fraction managed with diuretic and beta-blocker therapy — an inpatient level of care with high-complexity decision-making is clinically consistent and appropriate. Obesity in the context of decompensated or at-risk heart failure is a recognized driver of inpatient admission and directly influences the complexity of medical decision-making, fluid management strategy, and risk stratification. The breadth of her active conditions, including anemia and osteoporosis with a severely low T-score, further compounds clinical complexity and supports the high-complexity designation.

The payer's reliance on RARC N115 suggests a Local Coverage Determination was applied; however, no LCD applicable to initial inpatient evaluation and management services by a treating physician categorically excludes high-complexity care for a patient presenting with this degree of comorbidity. The denial appears to conflate the primary coded diagnosis with the totality of the clinical picture. The primary diagnosis code does not represent the sole basis for admission or the sole driver of complexity — the full constellation of active conditions documented in the medical record must be considered in any medical necessity determination.

We request that Absolute Total Care conduct a clinical review of the complete medical record for this encounter, including the documented active conditions and current medication regimen, which collectively substantiate both the inpatient level of care and the high-complexity designation of CPT 99223.

REQUESTED ACTION

MUSC Health respectfully requests that Absolute Total Care overturn the denial of Claim MUSC-20260413-E65A-1 and remit payment of \$2,212.20 for CPT 99223 as billed, consistent with the covered benefits under Member ID ABS643321435 and the applicable coverage period. Should the plan require additional clinical documentation, we ask that a specific written request be directed to our appeals team promptly so that any supplemental records may be provided within the appeal window. If this first-level appeal is not resolved in favor of payment, we reserve all rights to pursue further administrative and external review remedies available under the plan and applicable state and federal law.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record